

PATIENT DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Name: Patient 27

Age: 77 years old | Sex: Male

MRN: 000-TEST-27

Admission Date: [DATE] | Discharge Date: [DATE]

Admission Glucose: 487 mg/dL | HbA1c on admission: 9.2%

PRIMARY DIAGNOSIS & RELEVANT COMORBIDITIES

- Type 2 Diabetes Mellitus with acute hyperglycemic episode
- Anemia (iron-deficiency, Hgb 8.9 g/dL on admission)
- Chronic pain syndrome (lower back, bilateral knees)
- Hypertension
- Hyperlipidemia

HOSPITAL COURSE

Patient 27 presented with polydipsia, polyuria, and confusion. Initial labs revealed hyperglycemia (487 mg/dL) with mild metabolic acidosis (pH 7.28, bicarb 18 mEq/L). ICU monitoring was initiated for 24 hours with continuous glucose monitoring. IV insulin drip was administered and titrated per hyperglycemia protocol; patient transitioned to subcutaneous insulin on hospital day 2. Blood glucose levels stabilized to 140–220 mg/dL range by day 3. Hematology consulted; anemia workup initiated. Patient received 1 unit PRBCs on day 2 (post-transfusion Hgb 9.8 g/dL). Physical therapy evaluated for mobility; no acute orthopedic complications noted. Patient tolerated regular diet by discharge. Endocrinology was consulted on day 1 and provided insulin regimen optimization. Discharged in stable condition on hospital day 4.

DISCHARGE MEDICATIONS

| Medication | Dose | Frequency | Duration |

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| Insulin glargine (Lantus) | 20 units | q.d. at bedtime | indefinite |

Insulin lispro (Humalog)	8 units	AC meals (TID)	indefinite
Metformin XR	1000 mg	BID	indefinite
Lisinopril	10 mg	q.d.	indefinite
Atorvastatin	40 mg	q.d. at night	indefinite
Ferrous sulfate	325 mg	BID	3 months
Ibuprofen	400 mg	TID PRN	ongoing
Gabapentin	300 mg	TID	indefinite

GLUCOSE MONITORING & ACTIVITY

****HOME GLUCOSE MONITORING:**** Check blood glucose before meals and at bedtime daily (4x/day). Bring logbook to all endocrinology visits. Target range: 110–180 mg/dL fasting.

****ACTIVITY STATUS:**** Ambulate as tolerated. No heavy lifting >10 lbs. Physical therapy may resume at home per PT recommendations. Encourage light walking 20–30 min daily.

****FOOT CARE (CRITICAL):**** Inspect feet daily for blisters, ulcers, or erythema. Wear properly fitted shoes. Trim toenails straight across. See podiatry for any concerns. Patient 27 has intact protective sensation; continue annual foot exams.

DIETARY RESTRICTIONS

Diabetic diet, ADA 2000 kcal/day. Limit simple sugars and refined carbohydrates. Increase soluble fiber. Avoid sugary beverages. Encourage lean proteins and non-starchy vegetables.

FOLLOW-UP APPOINTMENTS

- ****Endocrinology Clinic:**** in 2 weeks (glucose review, insulin titration)
- ****Primary Care Physician:**** in 1 week (medication reconciliation, BP check)
- ****Hematology/Oncology:**** in